



ASHA workers educate women at the grassroots level about preventive healthcare (above); workers at a training session in Kohima (left)

MONITOR PROGRESS AGAINST STANDARDS

- Set IPHS standards.
- Conduct facility surveys.
- Independently monitor all committees at the block, district and state levels.

vergence for effectiveness and efficiency. In a way, it envisages people's involvement at the grassroots level. It has enlisted Accredited Social Health (ASHA) workers who are trained and paid by the centre. An ASHA worker is trained to handle health problems in her village such as post-delivery check-ups and post-immunisation. One of the aims of NRHM is to bring down infant mortality rate and maternal mortality rate.

Apart from these, the mission takes care to improve the existing medical infrastructure available at village and district levels while simultaneously working towards building new and modern infrastructure. For this, community health centres are being modernised and upgraded continuously. Even labour rooms are being modernised and we have got tremendous results in this direction. The programme has employed 2,000 char-



NRHM is not just another project. It is a seven-year mission that is very different from the projects we had till now.

DR ANBUMANI RAMADOSS

tered accountants, MBA graduates, data and IT professionals and accountants at district and state levels to track health-related information.

Q: What's the response to the mission?

A. The programme is not even two years old and already there are some very significant gains in the health sector. Reports from different states show great success and high acceptability rate of the mission. For instance, in Bihar, the average number of patients visiting a primary health centre every month has gone up from 39 in January 2006 to 3,015 in August 2006.

Reports also show significant increase in institutional deliveries because of demand of the financing scheme under Janani Surakhsha Yojana (JSY). From six lakh JSY cases in 2005-06, over 24 lakh beneficiaries have been covered under it with institutional deliveries being 21 lakh. The states have also put in place effective systems of procurement of drugs. Nearly 13,000 Rogi Kalyan Samitis and hospital development committees have been set

up. Also, 3,190 specialists, 3,494 doctors, 9,533 staff nurses, 17,736 ANMs and 17,903 paramedic staff have been appointed on contract and positioned during 2006-07. It is heartening that all states are cooperating fully with NRHM. I can proudly say that it is the country's biggest ever programme in the health sector. It is also one of the successful programmes of the UPA government.

Q: What are your other priority areas?

A. The experience of the past two years gives us confidence that we are on the right track and that we need to deepen institutional reforms and effective decentralisation through a concerted effort at capacity building. Apart from NRHM, two other programmes have been launched recently. They are National Programme on Prevention of Cardiovascular Diseases (Diabetes and Stroke) and National Programme on Speech and Hearing. We are also in the middle of setting up a National Drug Authority, which will be in place by next year and a new concept of a National Food Authority.